

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 11

Reviewing Action Plan & Consolidating Relapse Prevention

Fictional Educational Example — Full Detailed

SESSION INFORMATION

Client: RK (35 y/o Male)
Session #: 11 of 16

Therapist: Dr. A.
Focus: Action Plan review + Relapse prevention consolidation

PRE-SESSION THERAPIST NOTES

Client has been practicing consistently. Belief in the old perfectionistic thought remains low (~20%). He brought his written Action Plan. Today we will review the Action Plan in detail, refine the relapse prevention strategy, review recent experiments, and continue preparing him for the final phase of therapy with increased emphasis on self-management.

SESSION START & HOMEWORK REVIEW

Therapist: Good morning, RK. How has your week been? I see you brought your written Action Plan — great job.

Client: It was a good week. I used the Coping Card twice when I noticed worry creeping in. The experiments are still helping. I feel like I'm getting better at catching things early.

Therapist: That's excellent. Can you tell me about one time you used the Action Plan or Coping Card this week?

Client: On Wednesday I started over-preparing for a presentation and noticed my chest getting tight (early warning sign). I stopped, told myself I'd deal with it during Worry Time, and used the balanced belief. I still prepared reasonably but didn't go overboard. The presentation went fine.

Therapist: That's a perfect example of catching it early and using multiple tools. How did it feel compared to how you would have handled it a few months ago?

Client: Much better. Before, I would have spiraled and stayed up half the night. This time I handled it calmly and still did well.

REVIEWING THE ACTION PLAN

Therapist: Let's go through your Action Plan together and see if we need to refine anything.

Early Warning Signs (your list):

- Increased daily worry time
- Returning to over-checking and over-preparing
- Sleep disturbance starting again
- Avoiding meetings or social interactions

Top Coping Strategies (your list):

- Use thought record or balanced belief immediately
- Practice Worry Time / Postponement
- Do a quick behavioral experiment if fear feels strong
- Use problem-solving steps instead of worrying
- Talk to wife instead of withdrawing

Action Plan if symptoms return (your version):

1. Increase use of daily thought records and Worry Postponement for 1 week
2. Do at least one behavioral experiment related to the current fear
3. Review the Coping Card daily and rate mood and worry time
4. If symptoms do not improve after 7–10 days or start significantly affecting work or relationships → Contact therapist for a booster session

Therapist: This is a very clear and practical plan. Is there anything you want to add or change?

Client: I think it's good. Maybe add "Talk to wife early if I notice irritability returning" as one of the first steps.

Therapist: Good addition. We can update it together.

HOMEWORK ASSIGNMENT

Therapist: For this week:

1. Continue daily diary + Worry Time / Postponement
2. Practice using your updated Coping Card and Action Plan when warning signs appear
3. Do one more behavioral experiment
4. Write a short "Maintenance Plan" paragraph describing how you will keep using these tools after therapy ends

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed your excellent use of the tools and refined your relapse prevention plan. You're taking real ownership of this. How does having the full Action Plan written down feel?

Client: It feels solid. I now have a clear roadmap for what to do if things get difficult again.

Therapist: That's exactly what we want. Any questions?

Client: No, I'm clear.

POST-SESSION THERAPIST NOTES

- Client demonstrating strong self-management and ownership of relapse prevention tools.
- Action Plan and Coping Card are practical and personalized.
- Belief in old core thought remains low and stable.
- Next session: Review Maintenance Plan paragraph, continue experiments, deepen relapse prevention (booster session details), prepare for final sessions.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.